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RADIANTCARE PHYSICIANS PLLC
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LACEY, WA. 98503-1010

November 25, 2023

Updates to your agreement effective March 1, 2024

Dear RADIANTCARE PHYSICIANS PLLC:

Thank you for the care you provide to our members and your participation in our networks. Enclosed are documents updating your Regence BlueShield agreement, which will be effective March 1, 2024. These documents supersede the requirements in any other agreement documents. Please review these documents in their entirety:

- **Washington state addendum:** The addendum includes Washington state regulatory requirements.
- **Attachment D, Provider Dispute Resolution Process:** This updated document corrects outdated mailing addresses and updates references of our Fraud and Abuse department to our Special Investigations Unit. The Appeals for Providers section of our *Administrative Manual* has been updated to include this information. The updated section is available on our provider website at **regence.com**: Library>Administrative Manual.
- **Provider Network Addendum (PNA):** As a participating provider, if you do not currently participate in the Individual and Family and RealValue networks, we are expanding your network participation to include these networks, effective March 1, 2024. Enclosed is a non-signatory amendment to your Regence BlueShield provider agreement that replaces your previous *Provider Network Addendum* (PNA). By receiving this PNA, you will automatically become an in-network provider for the networks listed above, effective March 1, 2024. Learn more about our networks on our provider website: Contracting & Credentialing>Provider Networks.

Contact us

For questions that cannot be answered using Availity Essentials or our Self-Service Tool, call our Provider Contact Center at 1 (800) 253-0838. Thank you for your valued and continued participation in the plans we offer in your community.

Sincerely,

Denise Corcoran
Vice President, Network Management

**AMENDMENT TO
REGENCE BLUESHIELD
PARTICIPATING AGREEMENT**

THIS AMENDMENT, effective **March 01, 2024**, hereby amends the following agreements (collectively, the "Agreements" and each individually, the "Agreement") and any and all amendments, addenda, attachments or exhibits thereto by and between **Regence BlueShield** (Regence or as otherwise defined in the Agreement and hereafter referred to as "Regence") and **Provider**:

Contracting Hospital Agreement
Dental Provider Agreement
Independent Practice Association Agreement
Independent Physicians Association Agreement
Medical Equipment Vendor Agreement
Medical Group Agreement
Medical Services Agreement
Medicare Advantage Participating Ancillary Provider Agreement
Medicare Physician and Other Health Care Professional Agreement
Participating Ambulatory Surgery Center
Participating Ambulatory Surgery Center Agreement
Participating Ancillary Agreement
Participating Ancillary Provider Agreement
Participating Chiropractic Clinic Agreement
Participating Clinical Laboratory Agreement
Participating Dental Provider Agreement
Participating Dental Provider Agreement for Dental Clinics
Participating Dental Provider Agreement for Dental Groups
Participating Emergency Medical Services Agreement
Participating Facility Agreement
Participating HME Agreement
Participating Hospital Agreement
Participating Laboratory Agreement
Participating Medical Equipment Agreement
Participating Medical Equipment Vendor Agreement
Participating Medical Group Agreement
Participating Medical Services Agreement
Participating Organizational Provider (Facility) Agreement
Participating Organizational Provider (Laboratory) Agreement
Participating Physician and Other Health Care Professional Agreement
Participating Physician or Other Healthcare Professional Agreement
Participating Practitioner Agreement
Participating Professional Services Agreement
Participating Provider Agreement
Participating Services Agreement
Participating Technical Medical Facility Contract
Participating Vision Clinic Agreement
Participating Vision Practitioner Agreement
Physician and Other Health Care Professional Agreement
Physician Hospital Organization
Professional Services Agreement

All references herein to "Provider" shall mean "Physician or Other Health Care Professional," "Provider," "Practitioner," "Medical Group," "Medical Group Provider," "Hospital," "Facility," "Dental Clinic," "Dental Group," "Dental Provider," or as otherwise defined in the Agreement(s).

RECITALS

WHEREAS, Regence and Provider entered into the Agreement, wherein Provider agreed to provide Covered Services to Members; and

WHEREAS, Regence and Provider agree to continue their contract to provide health care services to Members, and amend the Agreement pursuant to the amendment with notice clause in the Agreement for the purpose of incorporating the provisions set forth below; and

NOW, THEREFORE, in consideration of the foregoing premises and other good and valuable consideration, the receipt and sufficiency of which are hereby acknowledged, the parties agree as follows:

AGREEMENT

1. In the event that a Washington State Addendum exists in the contract, it is hereby deleted in its entirety and replaced with the enclosed, revised Washington State Addendum.
2. The Attachment D, Provider Dispute Resolution Process is hereby deleted in its entirety and replaced with the enclosed, revised Attachment D, Provider Dispute Resolution Process.
3. The Provider Network Addendum is hereby deleted in its entirety and replaced with the enclosed, revised Provider Network Addendum.

Any term not defined herein shall have the meaning set forth in the Agreement.

To the extent that there is any conflict between the terms of the Agreement and the terms of this Amendment, the terms of this Amendment will prevail.

Except as expressly amended and supplemented by this Amendment, all other terms and conditions of the Agreement shall remain in full force and effect.

REGENCE BLUESHIELD



Vice President, Network Management

PARTICIPATING PROFESSIONAL SERVICES AGREEMENT WASHINGTON STATE ADDENDUM

This Addendum to the Participating Professional Services Agreement (the "Agreement") is entered into and made part of the Agreement with **Provider**, to recognize additional provisions that apply to Member Contracts sponsored, issued or administered by, or accessed through Regence that may be subject to regulation under Washington law; and for which Washington laws may control.

Regence and Provider each agree to be bound by the terms and conditions contained in this Addendum. In the event of a conflict or inconsistency between this Addendum and any term or condition contained in the Agreement, this Addendum shall control. This Addendum will be deemed to be updated to incorporate any changes to the laws and regulations referenced herein, effective as of the date of such changes. Except as specifically amended herein, all terms and conditions of the Agreement remain in effect.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

- 1.7 MEDICALLY NECESSARY or MEDICAL NECESSITY:** Except for services subject to WAC 284-43-7000, "Medically Necessary" or "Medical Necessity" means health care services or supplies that a physician or other health care professional, exercising prudent clinical judgment, would provide to a patient for the purpose of preventing, evaluating, diagnosing, or treating an illness, injury, disease, or its symptoms, and that are (a) in accordance with generally accepted standards of medical practice; (b) clinically appropriate, in terms of type, frequency, extent, site, and duration, and considered effective for the patient's illness, injury, or disease; (c) not primarily for the convenience of the patient, physician, or other health care professional; and (d) not more costly than an alternative service or sequence of services or supplies that are at least as likely to produce equivalent therapeutic or diagnostic results as to the diagnosis or treatment of that patient's illness, injury, or disease. For these purposes, "generally accepted standards of medical practice" means standards that are based on credible scientific evidence published in applicable peer-reviewed medical literature generally recognized by the relevant medical community, physician specialty society recommendations, and the views of physicians and other health care professionals practicing in relevant clinical areas and any other relevant factors. For services subject to WAC 284-43-7000, "Medical Necessity" or "Medically Necessary" shall have the meaning set forth in WAC 284-43-7010.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

- 3.1 PAYMENT.** Regence or Payor will compensate the Provider for Medically Necessary Covered Services provided to Members in accordance with this Agreement and Regence's or Payor's claims payment policies. Provider will have the right to audit Regence's records related to adjudication of Provider's claims. The audit may be performed either by the Provider or by an independent auditor selected by the Provider. Provider or the independent auditor will conduct audits during Regence's regular business hours at Regence's office and the audit will be limited to records necessary to perform the audit. The Provider will give Regence no less than three (3) business days advance notice of such claims audit and will inform Regence of the claim records to be audited. Regence will have the records for that time period available for the auditors at the time of the audit. Provider or the independent auditor will conduct the audits in a manner that, to the greatest extent possible, avoids disruption of Regence's business affairs and minimizes the burden on Regence. In conducting the audits, Provider or the independent auditor will comply with all laws, statutes, and regulations pertaining to the confidentiality of Member records. Provider or independent auditor will conduct the audit according to Regence's most current external audit guidelines. This Section will survive termination of this Agreement.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

- 4.2 COMPLIANCE WITH POLICIES AND GUIDELINES.** Provider will comply with rules, guidelines, policies, and procedures, including adherence to Bree Collaborative recommendations where available, whether outlined in this Agreement, or Provider Web Site. To the extent of any inconsistency between this Agreement and the Provider Web Site, this Agreement shall control. Regence may revise the rules, guidelines, policies, and procedures with sixty (60) days' notice. If

Provider objects to a change in rules, guidelines, policies, and procedures on the Provider Web Site, Provider may elect to terminate this Agreement pursuant to Article VII of this Agreement.

4.2.1 Regence's process to request an exception to the drug utilization management process can be found on the Provider Web Site.

4.2.2 In addition to compliance with other regulation applicable to the provision of virtual care, audio-only telemedicine billing requires advance consent from the Member and an "Established relationship" with the Member in accordance with RCW 48.43.735.

"Established relationship" means the provider providing audio-only telemedicine has access to sufficient health records to ensure safe, effective and appropriate care services and:

For health care services included in the essential health benefits category of mental health and substance use disorder services, including behavioral health treatment:

(A) The covered person has had, within the past three years, at least one in-person appointment, or at least one real-time interactive appointment using both audio and video technology, with the provider providing audio-only telemedicine or with a provider employed at the same medical group, at the same clinic, or by the same integrated delivery system operated by a carrier licensed under chapter 48.44 or 48.46 RCW as the provider providing audio-only telemedicine; or

(B) The covered person was referred to the provider providing audio-only telemedicine by another provider who has had, within the past three years, at least one in-person appointment, or at least one real-time interactive appointment using both audio and video technology, with the covered person and has provided relevant medical information to the provider providing audio-only telemedicine;

For any other health care service:

(A) The covered person has had, within the past two years, at least one in-person appointment, or, until July 1, 2024, at least one real-time interactive appointment using both audio and video technology, with the provider providing audio-only telemedicine or with a provider employed at the same medical group, at the same clinic, or by the same integrated delivery system operated by a carrier licensed under chapter 48.44 or 48.46 RCW as the provider providing audio-only telemedicine; or

(B) The covered person was referred to the provider providing audio-only telemedicine by another provider who has had, within the past two years, at least one in-person appointment, or, until July 1, 2024, at least one real-time interactive appointment using both audio and video technology, with the covered person and has provided relevant medical information to the provider providing audio-only telemedicine.

Failure to obtain member consent could result in disciplinary action against the Provider.

The referenced section of the Agreement is hereby added its entirety with the following:

4.13.2.5 Provider's Right to Audit. Provider shall have the right to audit Regence's records related to adjudication of Provider's claims pricing. The audit may be performed either by the Provider or by an independent auditor selected by the Provider. Such audits shall be conducted during Regence's regular business hours at Regence's office and shall be limited to records necessary to perform the audit. Provider shall give Regence no less than three (3) business days advance notice of such claims audit and shall inform Regence of the claim records to be audited. Regence shall have the records for that time period available for the auditors at the time of the audit. Such audits shall be

conducted in a manner that, to the greatest extension possible, avoids disruption of Regence's business affairs and minimizes the burden on Regence. Audits will comply with all laws, statutes and regulations pertaining to the confidentiality of Member records. Any audit shall be conducted according to Regence's most current external audit guidelines. These rights shall survive termination of this Agreement.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

4.16 PROVIDER-PATIENT RELATIONSHIP. Providers will maintain the provider-patient relationship with Members. Regence may not in any way preclude or discourage their Providers from informing Members of the care they require, including various treatment options, and whether in their view such care is consistent with Medical Necessity, medical appropriateness, or otherwise covered by the Member Contract. No health carrier may prohibit, discourage, or penalize a Provider otherwise practicing in compliance with the law from advocating on behalf of a Member with a health carrier. Nothing in this section shall be construed to authorize Providers to bind health carriers to pay for any service. No health carrier may preclude or discourage Members or those paying for their coverage from discussing the comparative merits of different health carriers with their providers. This prohibition specifically includes prohibiting or limiting providers participating in those discussions even if critical of a carrier.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

4.21.1 Member Hold Harmless. Provider hereby agrees that in no event, including, but not limited to, nonpayment by Regence, Regence's insolvency, or breach of this Agreement, will Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against a Member or person acting on their behalf, other than Regence, for services provided pursuant to this Agreement. This provision does not prohibit collection of Deductibles, Coinsurance, Copayments, and/or payment for Non-Covered Services, which have not otherwise been paid by a primary or secondary carrier in accordance with regulatory standards for coordination of benefits, from Members in accordance with the terms of the Member's Member Contract.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

4.21.2 Continue Providing Services. Provider agrees, in the event of Regence's or Payor's insolvency, to continue to provide Covered Services promised in this Agreement to Members of Regence or Payor under the Provider's care for the duration of the period for which premiums on behalf of the Member were paid to Regence or Payor or until the Member's discharge from inpatient facilities; whichever time is greater. The provision of such services and the payment to Provider for these services will be subject to the applicable terms of this Agreement and on the same basis as those services provided prior to insolvency.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

4.21.3 Member Contract. Notwithstanding any other provision of this Agreement, nothing in this Agreement shall be construed to modify the rights and benefits contained in the Member Contract.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

4.21.4 Charges to Members. Provider may not bill the Member for Covered Services, except for Deductibles, Coinsurance or Copayments where Regence or Payor denies payments because the Provider or facility has failed to comply with the terms or conditions of this Agreement. As required by WAC 284-170-421(4), Regence hereby informs Provider that willfully collecting or attempting to collect an amount from a Member knowing that the collection is in violation of this Agreement constitutes a class C felony under RCW 48.80.030(5).

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

4.21.5 Survival of Termination. Provider further agrees that (i) the above provisions will survive termination of this Agreement regardless of the cause giving rise to termination and shall be construed to be for the benefit of the Members, and (ii) this provision supersedes any oral or written contrary agreement now existing or hereafter entered into between the Provider and Member or persons acting on their behalf.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

4.21.6 Provider Contracts with Other Health Care Professionals. If Provider contracts with other health care providers who agree to provide Covered Services to Members of Regence or Payor with the expectation of receiving payment directly or indirectly from Regence or Payor, such providers must agree to abide by the above provisions.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

5.3 PROMPT PAYMENT OF CLAIMS. Regence will pay claims as soon as practical, but subject to the following minimum standards: (1) Regence will pay ninety-five percent (95%) of the monthly volume of clean claims within thirty (30) days of receipt by Regence; and (2) Regence will pay or deny ninety-five percent (95%) of the monthly volume of all claims within sixty (60) days of receipt by Regence. For purposes of this Section, a "clean claim" means a claim that has no defect or impropriety, including any lack of any required substantiating documentation, or particular circumstances requiring special treatment that prevent timely payments from being made on the claim in accordance with applicable Washington law. If Regence requires additional information to process a claim, Provider will provide such information at no charge. Once the additional information is received by Regence, Regence will process the claim in accordance with the above standards. Regence will communicate denial of a claim to Provider, including the specific reason why the claim was denied. If the denial is based upon Medical Necessity or similar grounds, then upon Provider's request, Regence will promptly disclose the supporting basis for the decision.

To the extent required by Washington law, Regence or Payor will pay simple interest at the rate of one percent (1%) per month on payable clean claims not paid within sixty-one (61) days of receipt. Such interest will be calculated based on the amount to be paid to Provider for the Covered Services and will be added to Provider's payment without further claim submission from Provider. Notwithstanding the foregoing, neither Regence nor Payor will be subject to interest, penalties, or late fees that may be established by Washington state law for clean claims for Covered Services provided to Members pursuant to a Member Contract with, or on behalf of, the federal government or otherwise pursuant to federal law, including, but not limited to, those under the Federal Employees Health Benefits Program, Medicare, and self-funded health plans.

These prompt payment of claims standards do not apply to claims about which there is substantial evidence of fraud or misrepresentation by Provider or Members or instances where Regence has not been granted reasonable access to information under Provider's control.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

5.5.1 Overpayments. Regence or Payor may request a refund from Provider of a payment previously made to satisfy a claim within twenty-four (24) months after the date that the payment was made or, in the case of a claim involving the coordination of benefits, within thirty (30) months of such date. Any such request must be in writing and must specify why Regence or Payor believes that Provider owes the refund. In the case of a claim involving the coordination of benefits, the request must include the name and mailing address of the other entity that has primary responsibility for payment of the claim. If Provider fails to contest a refund request in writing to Regence or Payor within thirty (30) days of receiving the request, the request is deemed accepted and Provider must pay the refund within thirty (30) days after the request is deemed accepted. If Provider has not paid the refund within thirty (30) days after the request is deemed accepted, Regence or Payor may recover the amount through an offset to a future claim. The parties agree that this Section does not apply in cases of fraud.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

5.5.2 Refunds and Adjustments. Provider may request additional payment from Regence or Payor to satisfy a claim within twenty-four (24) months after the date that the claim was denied or payment intended to satisfy the claim was made or, in the case of a claim involving the coordination of benefits, within thirty (30) months of such date. Any such request must be in writing and must specify why Provider believes that Regence or Payor owes the additional payment. In the case of a claim involving the coordination of benefits, the request must include the name and mailing address of any entity that has disclaimed responsibility for payment of the claim. The original claims decision will be final and binding unless Provider requests additional payment within the twenty-four (24) or thirty (30) month time periods, as applicable. The parties agree that this Section does not apply in cases of fraud.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

8.2 INTERNAL PROVIDER APPEAL PROCESSES. Regence's provider appeal processes ("Provider Dispute Resolution Process") in its entirety are set forth in Attachment D hereto.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

8.3 POST-APPEAL PROCESSES. Regence's provider appeal processes ("Provider Dispute Resolution Process") in its entirety are set forth in Attachment D hereto.

The referenced section of the Agreement is deleted in its entirety and replaced with the following:

9.11 GOVERNING LAW / VENUE. This Agreement is governed by the laws of the State of Washington, without giving effect to any conflict-of-law principle that would result in the laws of any other jurisdiction governing this Agreement. Any action, suit, or proceeding arising out of the subject matter of this Agreement will be litigated in courts located in King County, Washington. Each party consents and submits to the jurisdiction of any local, state, or federal court located in King County, Washington.

ATTACHMENT D PROVIDER DISPUTE RESOLUTION PROCESS

This Dispute Resolution Process contains information about the processes for the following types of provider appeals and disputes:

- a. Dental Provider Appeals and Disputes
- b. Medical Provider Appeals and Disputes
- c. Hospital/Facility Appeals and Disputes

I. DEFINITIONS

- 1.1 Adverse Determination (Appeal):** For the purpose of the Provider appeal process, adverse determination means any of the following: a denial, reduction, or termination of, or a failure to provide or make payment (in whole or in part) for, a benefit, including any such denial, reduction, termination, or failure to provide or make payment that is based on a determination of any of the following:
- a. Application of utilization review;
 - b. Determination that a treatment is not Medically Necessary; or
 - c. Denials related to upcoding (including DRG);
 - d. Application of a Current Procedural Terminology (CPT®) modifier, and/or other reassignment of a code by us to patient specific factual situations, including the appropriate payment when two or more CPT Codes are billed together, or
 - e. Whether a payment enhancing modifier is appropriate.
- 1.2 Appeal Record:** Includes all information which was relied upon in making the payment determination; or was submitted, considered, or generated in the course of making the payment determination, whether or not such document, record, or other information was relied upon in making the payment determination; or demonstrates compliance with our claims procedures, administrative processes and safeguards; or constitutes a statement of policy or guidance with respect to the payment determination.
- 1.3 Claims:** A Provider's request for payment submitted in the usual course of business between the Provider and us.
- 1.4 Dispute:** For the purpose of the Provider dispute process, dispute means any of the following: a denial, reduction, or termination of, or a failure to provide or make payment (in whole or in part) for, a benefit, including any such denial, reduction, termination, or failure to provide or make payment that is based on a determination of any of the following:
- a. Failure to secure Preauthorization
 - b. Failure to notify of Inpatient Admission
 - c. Determination that services are related to a Hospital Acquired Condition
 - d. Determination that records do not support billing during a Hospital Bill Review (Line Item Review)
 - e. Any other dispute that does not meet the definition of Adverse Determination outlined in the Definitions
- 1.5 External Review:** Review of an Adverse Determination Appeal submitted to the External Review Organization with which we have contracted to provide these review services by a provider in compliance with the terms of the Adverse Determination Appeal Process.
- 1.6 External Review Organization ("ERO"):** An independent organization employing providers and other medically qualified individuals or experts, which acts as the decision maker for External Reviews, through an independent contractor relationship with us.
- 1.7 "Our" or "We":** References to our or we mean Regence or the Company.

- 1.8 **Provider:** Provider means provider, practitioner, physician, medical group, clinic, hospital or other health care professional as defined in the Agreement.
- 1.9 **Provider Appeal:** Formal request from a contracted Provider to reconsider an Adverse Determination when the Provider is at financial risk for the services.
- 1.10 **“You” or “Your”:** References to you or your mean Provider as defined in the Agreement.

II. **MEDICAL, DENTAL AND HOSPITAL PROVIDER ADVERSE DETERMINATION APPEAL AND DISPUTE PROCESS**

- 2.1 Unless otherwise indicated herein, the Participating Provider must exhaust the applicable provider appeals process before initiating any of the post-appeal processes set forth herein.

If the Participating Provider submits a dispute to the internal provider appeal process, and Regence fails to render a timely decision based on the time frames described in the Agreement, Provider may bypass the provider appeal process and proceed directly to one or more of the post-appeal processes described below.

Notwithstanding, this is not intended to limit access to judicial remedies or relief.

- 2.2 **Applicability.** Our Provider Adverse Determination Appeal and Dispute Process will apply when the Provider is at financial risk for the cost of the claim. The member appeal process will apply when the member is or may be at financial risk for the cost of the claim.

The following are not eligible under the Adverse Determination Appeal or Dispute Process:

- a. Appeals made by non-contracted providers. Appeals by non-contracted providers may be eligible for the member appeal process.
- b. The member has filed suit under Section 502 of ERISA or other suit for denial of the health care services or supplies regarding an Adverse Determination.

2.3 **Process for Submission of all levels of Adverse Determination Provider Appeals or Disputes**

- a. Use the *Provider Appeal Form*, which can be found on our provider website at **regence.com**.
- b. The completed appeal form or a written description of the issue(s) on the appeal may be submitted via the electronic portal, or the mailing address or fax number listed in the Adverse Determination Provider Appeals or Disputes section on the Contact Us page of the Provider Web Site.
- c. The following information must be submitted with the *Provider Appeal Form* or the written description of the issue(s) on appeal:
 - i. A detailed description of the disputed issue(s);
 - ii. The basis for disagreement with the decision; and
 - iii. All evidence and documentation supporting your position.

III. **PROVIDER FIRST LEVEL (INTERNAL) APPEALS AND DISPUTES**

- 3.1 **Time Period for Submission of a Level One Adverse Determination Appeal or Dispute by Provider.** An Adverse Determination Provider Appeal or Dispute must be submitted in writing (with all applicable evidence/documentation to justify an overturn of the original denial) within the following timeframes:

- a. Within 24 months after payment of the claim or notice that the claim was denied or 30 months for claims subject to coordination of benefits.

- b. If a Provider wishes to appeal a refund request initiated by us, you can submit an Appeal within the same timeframe as listed above. Note: The timeframe begins when the written request for refund is sent to the Provider.

Failure to request review with all applicable documentation within the stated time period will preclude the right to appeal and may jeopardize the right to contest the decision in any forum.

3.2 Level One Internal Appeal and Dispute Review Provisions.

- a. The individual reviewing the issues(s) of the appeal/dispute will meet the following criteria:
 - i. Is not an individual who made or consulted in the initial determination;
 - ii. Is not a subordinate of an individual involved in the initial determination.
- b. If your appeal/dispute determination is unfavorable in whole or in part, we will communicate a written decision on an Internal Review of an Adverse Determination Appeal within thirty (30) calendar days of our receipt of all documentation reasonably needed to make the determination. A description of the External Review option will be supplied with the written decision, including the time limit for requesting External Review.
- c. Additionally, for unfavorable determinations in whole or in part, you have the option to seek a Level Two appeal/dispute. A description of the Level Two Review option will be supplied, including the time limit for requesting a Level Two Review which is ninety (90) calendar days after the written Internal Review determination.

3.3 Qualified Reviewer. For an Internal Review of decisions involving Utilization Review or whether a treatment is Medically Necessary only a medical or dental provider holding an active, unrestricted license, who possesses the appropriate level of training and/or expertise required to evaluate the necessity of the service under review, and who is other than the one that made the initial Adverse Determination, may deny the appeal of the Provider who treated the condition. A nurse or other health care professional employed by us may review the Adverse Determination Appeal and may grant but not deny it. If the nurse or other health care professional does not grant the appeal, then a Qualified Reviewer, designated by us, other than the one that made the initial Adverse Determination, shall review and decide the Adverse Determination Appeal in accordance with our medical and reimbursement guidelines.

IV. PROVIDER SECOND LEVEL (INTERNAL) APPEALS AND DISPUTES:

4.1 Level Two Adverse Determination Appeal (External Review) process can be found in Article VII, Post Appeal Process.

4.2 Level Two Disputes (Internal Review).

- a. If the initial determination is upheld through the Internal Review process, the option to seek a second level Internal Review of the determination will be made available for Disputes.
- b. Prerequisites for a Level Two Disputes (Internal) Review:
 - i. The Level One Internal Review process must be exhausted before requesting a Level Two Internal Review.
 - ii. Provider must submit a written notice to us within ninety (90) calendar days from the date of the Level One Internal Review Determination Letter.
 - iii. The appeal meets the definition of a Dispute as described in the Definitions listed above. These disputes include:
 - 1. Failure to secure Preauthorization
 - 2. Failure to notify of Inpatient Admission
 - 3. Determination that services are related to a Hospital Acquired Condition
 - 4. Determination that records do not support billing during a Line Item Audit

- iv. Any other dispute that does not meet the definition of Adverse Determination outlined in the Definitions listed above.
- v. Only one Level Two review is available. If a Provider utilizes a second external level review process outlined above, a second internal review will not be made available.

4.3 Level Two Dispute (Internal) Review Provisions.

- a. The individual reviewing the issue(s) on appeal will meet the following criteria:
 - i. Is not an individual who made or consulted in the initial determination;
 - ii. Is not a subordinate of an individual involved in the initial determination.
- b. If your appeal determination is unfavorable in whole or in part, we will communicate a written decision on a Second Level Internal Review of the Dispute within thirty (30) calendar days of our receipt of all documentation reasonably needed to make the determination. This is the final level of dispute.

The Provider has the option to request an in-person meeting. If no request for an in-person meeting is made, the process for review outlined above will proceed without one. If an in-person meeting is requested, the meeting will be held within forty-five (45) calendar days of the written request. If your appeal determination is unfavorable in whole or in part, we will communicate a written decision on a Second Level Internal Review of the Dispute within thirty (30) calendar days of the in-person meeting. This is the final level of internal dispute review process.

V. SPECIAL INVESTIGATIONS UNIT APPEAL PROCESS

- 5.1** The Special Investigations Unit Appeal Process is intended to give you an opportunity to request reconsideration of audit findings issued by our Special Investigations Unit and to ensure we have reviewed all information relevant to the audit findings. Please note that contract terminations resulting from audit findings must follow the Provider Contract Termination Appeal Process.
- 5.2 Level One Appeal.** Upon receipt of our audit findings, you have forty-five (45) business days to review and dispute these findings before the audit becomes final. In order to appeal the findings, you must submit a written request for appeal. You will be given the address for where to send your request.

The request must be received by us within forty-five (45) business days of your receipt of the audit findings and must include, at a minimum, the following:

- a. A detailed statement of the issue(s) in dispute;
- b. At the election of the Provider, notification of a request for a meeting with the person(s) reviewing the issue(s) in dispute;
- c. Any documents which the Provider contends supports your position

(Exception: Please note that all documentation required to justify your billing, including but not limited to chart notes, must be present in your files at the time of an audit. Additions to file documentation and/or the production of files that were not made available to us at the time of an audit will not be considered in connection with an appeal involving adverse audit findings. We will, however, consider your explanation as to why the documentation was not present at the time of the audit.)

If we do not receive such a request for appeal within forty-five (45) business days of your receipt of the audit findings, the findings will be final.

The Level One Appeal decision will be made by a manager of our Special Investigations Unit, and our other Regence representatives, as determined by the manager. At the discretion of the manager, one of our Medical Directors may be consulted prior to the final decision.

A meeting, prior to the review of your request for reconsideration, can be arranged either at your office, at our office, or by telephone, as mutually convenient. You must request this meeting when submitting your request for appeal.

At the meeting, you may appear in person and may be accompanied by an attorney or other representative. The purpose of this meeting is to give you an opportunity to present your position prior to the appeal determination.

If additional documentation to reach a decision is needed, the additional documentation must be submitted within twenty (20) calendar days of the date of our written request for information, unless your written request for a reasonable extension of time is granted.

If the requested documentation is received on time, it will be included in the request for appeal.

If the documentation is not received on time, the appeal will continue and a decision will be made based on the information originally submitted.

During the period of time in which we are waiting for additional information, the appeal decision time frame is suspended until the information is received or the time to respond to the request has expired.

You will be sent written notice of the decision within forty-five (45) business days following the meeting or, if no such meeting was requested, within forty-five (45) business days of our receipt of the Level One Appeal.

The decision on a Level One Appeal is deemed final forty-five (45) business days after your receipt of our decision, unless a timely written request for a Level Two Appeal is received.

5.3 Level Two Appeal. If you are not satisfied with the decision made following the first request for appeal, you may request a second appeal of the audit findings. The written request for a Level Two Appeal and any supporting information, must be received by us within forty-five (45) business days of your receipt of our decision. The address where to send your request will be included in our response to your Level One Appeal.

Level Two Appeals are categorized as either a clinical dispute or a non-clinical dispute.

For clinical disputes, a Medical Director review will be held no more than forty-five (45) business days following receipt of the request, not including the time in which we are waiting for additional information from you. The review will be conducted by a Medical Director who was not involved in an earlier review of the audit findings.

For non-clinical (administrative/contractual) disputes, a decision will be made by a manager of the Special Investigations Unit not involved in an earlier review of the audit findings, and/or other Regence representatives, as deemed by the manager.

If additional documentation is needed to reach a decision, the additional documentation must be submitted within twenty (20) calendar days of the date of our written request for information, unless your written request for a reasonable extension of time is granted.

- a. If the requested documentation is received on time, it will be included in the request for appeal.
- b. If the documentation is not received on time, the appeal will continue and a decision will be made based on the information originally submitted.

During the period of time in which we are waiting for additional information, the forty-five (45) business day clock is suspended until the information is received or the time to respond to the request has expired.

You will be sent written notice of the decision within forty-five (45) business days following the Medical Director review, for a clinical dispute, or forty-five (45) business days following receipt of the Level Two appeal, for a non-clinical dispute.

The Level Two Appeal is the final step in the Special Investigations Unit Appeal Process. Once a decision has been made, the Special Investigations Unit Appeal Process has been completed and the decision shall be deemed final. If you are not satisfied with our decision after completing the Special Investigations Unit Appeal Process and want to continue to dispute the issue(s), you must initiate the appropriate process(es) as outlined in your Provider contract.

VI. PROVIDER CONTRACT TERMINATION APPEALS

6.1 Provider Contract Termination Appeals. A contracted Provider may initiate an appeal of a contract termination decision made by us through the Provider Contract Termination Appeal Process.

6.2 Level One Appeal. To request a Level One Appeal, you must send a written request to the Credentialing Department, at the address listed in the Contact Us section of the Provider Web Site within thirty (30) business days of receipt of the termination notification.

A request for an appeal regarding a contract termination must include, at a minimum:

- a. A detailed description of the disputed issue(s)
- b. The basis for your disagreement with the decision
- c. All evidence and documentation supporting your position

(Exception: Please note that all documentation required to justify your billing, including, but not limited to, chart notes, to be present in your files at the time of an audit. Additions to file documentation or the production of files that were not made available to us at the time of an audit will not be considered in connection with an appeal involving adverse audit findings. We will, however, consider your explanation as to why the documentation was not present at the time of the audit.)

- d. Your requested outcome

Upon receipt of the Level One Appeal request, we will send you an acknowledgement letter within fifteen (15) business days.

The Level One Appeal Panel is comprised of at least three (3) individuals that have not been directly involved in the Credentialing Committee or termination decision and have the appropriate level of knowledge and training to understand the issues presented. At least one panel member must be a participating provider.

Level One Appeals meetings are held on a bi-monthly basis. Your appeal will be scheduled for review at the next available Level One Appeal Panel meeting, subject to the time your appeal request and any additional information are received and the volume of appeals being reviewed by the panel.

If additional information is requested, it must be submitted within fifteen (15) business days of the date of the written request for information, unless a written request for a reasonable extension of time is granted.

If the information is not received on time, a decision will be made at the next Level One Appeal panel meeting, based on the limited information available.

If the additional information is received on time, the new information will be taken into consideration at the next Level One Appeal panel meeting.

Information not submitted within the time limit will not be considered for the Level One Appeal, unless otherwise allowed by the Level One Appeal Panel.

You will receive a written determination within ten (10) business days of the Level One Appeal panel decision.

The Level One Appeal decision is deemed final on the thirtieth (30th) business day after you receive it, unless a written request for a Level Two Appeal is received timely.

6.3 Level Two Appeal “In-Person Hearing”. If you are not satisfied with the results of the Level One Appeal, you may submit a written request to the Credentialing Department, at the address listed in the Contact Us section of the Provider Web Site, for a Level Two Appeal, “in-person hearing” no later than thirty (30) business days after your receipt of the Level One Appeal decision.

The Level Two Appeal Panel is comprised of at least three (3) individuals that have not been directly involved in the Level One Appeal, the Credentialing Committee or the termination decision and have the appropriate level of knowledge and training to understand the issues presented. At least one panel member must be a participating provider.

The request for a Level Two Appeal must identify in detail the following:

a. All issues on which you request re-evaluation

b. Information not previously submitted to the Level One Appeal panel, if any

(Exception: We expect all documentation required to justify your billing, including, but not limited to, chart notes, to be present in your files at the time of an audit. Additions to file documentation or the production of files that were not made available to us at the time of an audit will not be considered in connection with an appeal involving adverse audit findings. We will, however, consider your explanation as to why the documentation was not present at the time of the audit.)

c. Your requested outcome

The hearing is generally completed within two (2) hours and will be scheduled for two (2) hours, unless you notify us when requesting your Level Two Appeal that additional time is needed. We will make our best efforts to accommodate reasonable requests for additional time, as long as we are notified when you request the Level Two Appeal.

Upon receipt of the Level Two Appeal request, we will send you an acknowledgement letter within fifteen (15) business days.

Level Two Appeals meetings are scheduled upon request. We will make our best efforts to provide proposed times and dates within sixty (60) business days of the Company’s receipt of your Level Two Appeal request. Once you have been provided the proposed times and dates you will have five (5) business days to notify us of your preferred time and date for the Level Two Appeal hearing. If you fail to notify us of your preferred time and date for the Level Two Appeal hearing within five (5) business days of receiving the proposed times and dates, the hearing will be set on one of the proposed times and dates.

Prior to the Level Two Appeal hearing, you will receive a “Notice of Hearing”. The “Notice of Hearing” will indicate the following:

i. Date of the hearing

ii. Time of the hearing

iii. Location of the hearing

- iv. Names of the members of the Level Two Appeal panel
- v. Reasons for the adverse action
- vi. Names of witnesses who will testify on our behalf at the hearing
- vii. Your rights at the hearing

At the hearing, you have the following rights:

1. To have representation by an attorney or other person of your choice
2. To have a court reporter make a record of the proceedings at an additional cost to you. Costs associated with the court reporter must be paid by you prior to receiving a copy of the transcript
3. To call witnesses and to examine/cross-examine witnesses
4. To present relevant evidence (as determined by the panel)
5. To submit a written statement at the close of the hearing

Approximately thirty (30) calendar days before the scheduled date of the hearing, a Level Two Appeal binder will be sent to you or your representative. The binder will include, among other things, the documentation reviewed by the Credentialing Committee initially and at the Level One Appeal, as well as any documentation submitted by you. If you wish to submit additional information to further supplement the Level Two Appeal binder, this information, as well as a list of witnesses that you plan to call, examine, and cross examine at the hearing, must be received no later than fourteen (14) calendar days prior to the hearing date. Unless otherwise allowed by the Level Two Appeal panel, documentation and witnesses not submitted at least fourteen (14) calendar days prior to the hearing date will not be considered by the Level Two Appeal panel and should not be brought to the hearing for the panel's consideration. The only exception is that you may submit a written statement at the close of the hearing.

If the Level Two Appeal binder is later supplemented with new or revised information prior to the hearing, you will receive copies of the new or revised material as soon as practicable before the scheduled date of the hearing. After the Level Two Appeal binder has been finalized, it will be forwarded to the Level Two Appeal panel for review prior to the hearing. Neither you nor we may supplement the binder within thirteen (13) calendar days prior to the hearing, unless a written request for an exception is approved by the Chair of the Level Two Appeal panel.

You will receive written notification of the Level Two Appeal decision within fifteen (15) business days of the hearing. If the Level Two Appeal panel cannot reach a decision within fifteen (15) business days, or if additional information is needed to reach a decision, you will be informed of any additional information needed and a new date by which the decision will be made.

Decisions of the Level Two Appeal panel related to contract terminations are deemed final. Once a decision has been made by the Level Two Appeal panel, you have completed the Provider Contract Termination Appeals process. If you are not satisfied with our decision after completing the Provider Contract Termination Appeal process and want to continue to dispute the issue(s), you must initiate the appropriate process(es) as outlined in your provider contract.

6.4 Additional Information Regarding the Provider Contract Termination Appeals Process.

a. Provider Status During a Contract Termination Appeal

You will continue as a participating provider; however, you will be temporarily removed from all provider directories and any pending action by us is put in abeyance until the appeal is resolved and a final decision is made. If, however, the basis for the termination decision relates to the health, safety or welfare of our members, or if we have exercised our right to

immediately terminate the provider contract for reasons allowed by the provider contract, your participation status will be terminated for the duration of the appeal process and reinstated only if you prevail during the Provider Contract Termination Appeal process.

b. The Data Bank Reportable Actions

We are required by law to report certain adverse actions or decisions against you to the Data Bank. If our termination decision stands, either by virtue of you choosing not to appeal or if the decision is upheld by the appeals panel, we may be obligated to report this termination to the Data Bank, as applicable. You may not “self-term” to avoid being reported to the Data Bank. Additional information on these reporting requirements is available on the Data Bank website, at npdb.hrsa.gov.

VII. POST-APPEAL PROCESS

7.1 POST-APPEAL PROCESSES. If, after the exhaustion of the applicable internal provider appeal process, either party is dissatisfied with the outcome of the internal provider appeal and wants to further dispute the issue(s), the disputed issue(s) must be submitted to one or more of the processes as described below. Any prerequisites to initiating one of the processes described below must be met before the process can be initiated.

7.1.1 External Review. For disputes that have exhausted the Billing Dispute and Medical Necessity/Investigational procedure appeal process, the Participating Provider may elect to resolve the disputed issue(s) by External Review, if certain conditions are met. A description of the External Review process and any prerequisites to initiating the External Review process can be found below. Disputes that do not meet the criteria to be submitted to External Review may be submitted to External Review only upon mutual written agreement of the parties.

7.1.1.1 Prerequisites for Level Two Adverse Determination Appeals (External Review):

- a. The Level One Internal Review process must be exhausted before requesting a Level Two External Review of the determination unless both we and the Provider agree in writing to forego the Internal Review process and proceed directly to External Review.
- b. A Provider who chooses External Review will submit a written notice to us within ninety (90) calendar days from the date of the Internal Review Determination.
- c. The appeal meets the definition of Adverse Determination as described in the Definitions listed above.
- d. Only one Level Two review is available. If provider utilizes a second level internal review process (for Facilities), a second level external review will not be made available.

7.1.1.2 Level Two Adverse Determination Appeals (External Review) Provisions.

- a. An Adverse Determination Appeal may be submitted for External Review. The request for External Review is at the option of the Provider, who may instead choose any other dispute resolution allowed by the Agreement.
- b. The Adverse Determination Appeal must be submitted to us in writing. The Administrative Manual and the Provider Appeal Form provides detailed contact information.
- c. We will forward Adverse Determination Appeals that meet the prerequisites as listed in section 7.1.1.1 above to a designated External Review Organization.

- d. The Provider shall pay a filing fee of \$50.00 for each Adverse Determination Appeal.
 - i. We shall notify you that the filing fee is due;
 - ii. Payment must be submitted before the External Review begins; provided, however, that you shall be entitled to a refund of such payment in the event that you prevail in the External Review process.
 - iii. You shall submit the filing fee within sixty (60) calendar days of notice from us that filing fee is due or the External Review request will be closed.
- e. Upon receipt of a timely filing fee, we will provide to the External Review Organization the Appeal Record.
- f. The External Review Organization will process the Adverse Determination Appeal and notify you and us of its recommendation within thirty (30) calendar days of receipt of the filing fee.
- g. In the event that the External Review decision requires payment by us, such payment shall be initiated within fifteen (15) calendar days after we received notice of the determination.

7.1.2 Mandatory Non-Binding Mediation. For disputes that have not been or cannot be submitted to External Review, the disputed issue(s) must be submitted to mandatory non-binding mediation prior to seeking litigation. Mandatory non-binding mediation must be requested within sixty (60) days following the date of Regence's decision on Provider's last internal provider appeal. Where Provider is allowed to bypass the internal provider appeal process as provided herein, mandatory non-binding mediation must be requested within sixty (60) days from the last day Regence has to timely respond to a dispute. Provider and Regence shall each bear their own costs of mediation and shall split equally the costs of the third-party mediator.

If the health carrier fails to grant or reject a request within thirty days after it is made, the complaining health care provider may proceed as if the complaint had been rejected. A complaint that has been rejected by the health carrier may be submitted to nonbinding mediation.

7.1.3 Litigation. If the parties are unable to resolve a dispute through nonbinding mediation, and if either party wishes to pursue the dispute, such party may commence litigation proceedings.

**REGENCE BLUESHIELD
PROVIDER NETWORK ADDENDUM
Effective: March 01, 2024**

This Addendum to the Agreement between Regence BlueShield (“Regence”) and **Provider** is to recognize network participation and additional provisions which apply to the networks. Except as specifically amended herein, all terms and conditions of the Agreement remain in effect.

WHEREAS, All references herein to “Provider” shall mean “Practitioner”, “Provider”, “Clinic”, or “Hospital” as those terms are used and defined in the Agreement; and

WHEREAS, Regence and Provider are parties to the Agreement, whereby Provider agrees to provide Covered Services to Members; and

WHEREAS, Regence desires Provider to be included in the networks indicated below pursuant to the terms of this Addendum; and

NOW, THEREFORE, in consideration of the covenants and agreements contained herein, and other good and valuable consideration, the receipt and sufficiency of which are hereby acknowledged, the parties hereto, intending to be legally bound, agree as follows:

I. PROVISIONS

- 1.1** This Addendum shall continue in effect, unless terminated according to the process set forth in the Agreement. Provider has the right to discontinue participation in any network(s) pursuant to the terms of the Agreement.
- 1.2** Provider shall admit or arrange for hospital admissions and referral services of Members only to network providers within their applicable network, unless the Member’s condition makes it impossible, the service is not available through the network, or the Member chooses care outside the network. Provider should advise the Member whenever health care services are to be obtained outside of their network that the Member may be subject to additional out-of-pocket expense. Any questions regarding network participation and benefit levels should be directed to Regence.
- 1.3** Provider agrees to accept the network payment set forth on the Reimbursement Schedule and Medicare Advantage Addendum, if applicable, as payment in full for Covered Services rendered to Members for networks in which Provider is participating as indicated below.

Provider is to participate in those networks indicated with a “Yes” below.

Participation	Commercial Networks
Yes	Participating
Yes	Preferred
Yes	Individual and Family Network
Yes	RealValue
	Blue High Performance Network
	Eastside Health Network
	MultiCare Connected Care
	UW Medicine
	Virginia Mason Franciscan Health
Yes	Option
Participation	MedAdvantage Networks
	SelectMed
Yes	Regence MedAdvantage PPO
Yes	Regence BlueAdvantage HMO